

RxKids Hawai'i - Cost & Benefits by Income Quintile, TY 2028

Statutory eligibility: Medicaid (clause 1) OR income <= 300% FPL incl. expected unborn child (clause 2)

Point estimate - true uncertainty is +/-30-40%. No admin caseload exists to calibrate against; cost hinges on soft pregnancy/infant incidence assumptions. Read with the assumption band.

Fiscal cost (annual)

Total program cost (benefit)	\$31,961,813
Prenatal arm	\$10,070,160
Postnatal arm	\$21,891,653
Administrative load (8%)	\$2,556,945
Appropriation total (benefit + admin)	\$34,518,758
Sampling 90% CI	\$24,247,071 - \$39,676,556
Assumption band	\$19,177,088 - \$40,751,920

Scenario comparison

Take-up held constant (0.90 newborn / 0.83 prenatal); only the eligibility gate and postnatal window change. Cost = annual benefit dollars; appropriation adds the admin load.

Statutory (Medicaid OR <=300% FPL) · 6-mo postnatal	\$4,500	\$31,961,813	\$34,518,758	14,011
Universal · 6-mo postnatal	\$4,500	\$53,259,558	\$57,520,322	23,347
Universal · 12-mo postnatal	\$7,500	\$89,738,707	\$96,917,803	23,347

Potential option - extend postnatal 6-12 months

The program may opt to extend postnatal payments to the full 12 months (Flint's upper bound). Priced here as an optional add-on:

Additional cost (+6 months)	\$21,891,653
Total cost (12-month design)	\$53,853,466

First fiscal year (launch) - 12mo operating, 12mo enrollment ramp

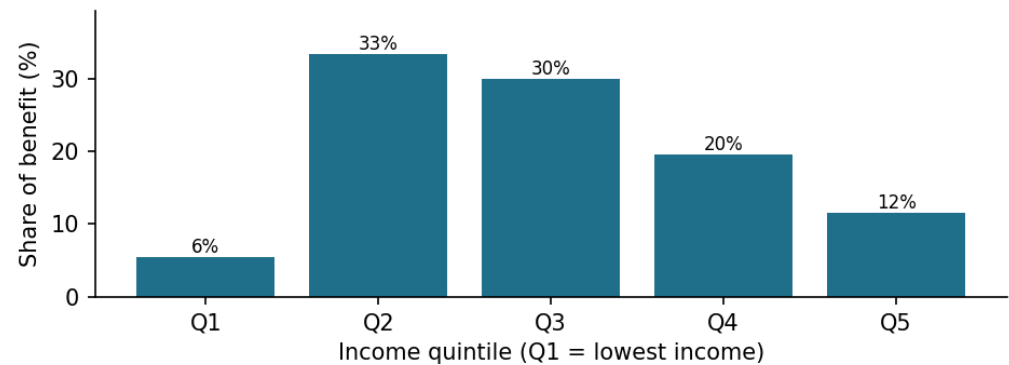
Year-1 cash disbursement, below steady state: enrollment ramps up and the 6-month postnatal caseload takes time to fill.

Year-1 disbursement (total)	\$13,258,639
Prenatal / Postnatal	\$5,454,670 / \$7,803,969
Postnatal deferred to next FY	\$4,054,010
Year-1 as % of steady state	41%
Ramp sensitivity (6 / 12 / 18 mo)	\$20,742,341 / \$13,258,639 / \$8,839,093

Household reach

Eligible families (weighted)	7,340
Expected recipients / year	14,011
Expected pregnancies / infants	6,713 / 7,297
Avg benefit per recipient	\$2,281

Benefit received by income quintile



Quintile	Avg income	Families	Recipients	Total benefit	Share
Q1	\$17,182	110,267	772	\$1,760,415	5.5%
Q2	\$57,024	110,263	4,679	\$10,673,739	33.4%
Q3	\$96,722	110,317	4,200	\$9,580,498	30.0%
Q4	\$154,809	110,231	2,739	\$6,247,330	19.5%
Q5	\$334,087	110,343	1,622	\$3,699,833	11.6%

Cost by county

County	Cost total	Prenatal	Postnatal	Recipients
Hawaii	\$6,382,030	\$2,010,777	\$4,371,253	2,798
Honolulu	\$21,904,984	\$6,901,570	\$15,003,414	9,602
Kauai	\$2,171,819	\$684,272	\$1,487,547	952
Maui	\$1,502,980	\$473,542	\$1,029,439	659

Both arms are driven by OBSERVED births (age-0 dependents in each PUMS tax unit), calibrated to CDC NVSR resident births and projected to the target year with the repo's damped-trend ensemble. Each eligible birth draws one prenatal + one postnatal payment. Cost = the take-up-adjusted benefit weighted by the household (WGTP) weight. Eligibility is tested on MAGI at the tax-unit (MAGI-household) grain, the family Medicaid uses. Benefits-by-quintile rank SPM units on summed income into population-equal fifths. Sampling CI is ACS sampling error (SDR, 80 replicate weights); the assumption band sweeps take-up and the birth count (+/-25%). 2026-2028 FPL is CPI-projected off the 2025 HHS table. Full methodology: RXKIDS_METHODODOLOGY.md.